Pertuzumab + trastuzumab · with selected chemotherapy

intravenous (IV) pertuzumab and IV trastuzumab every 21 days. Before surgery: 3–6 cycles alongside the selected chemotherapy. After surgery: up to 18 cycles overall under the agreed adjuvant plan. If an anthracycline is used first, finish it before starting these antibodies with the taxane phase. Confirm the exact companion regimen and local access.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Heart / breathing

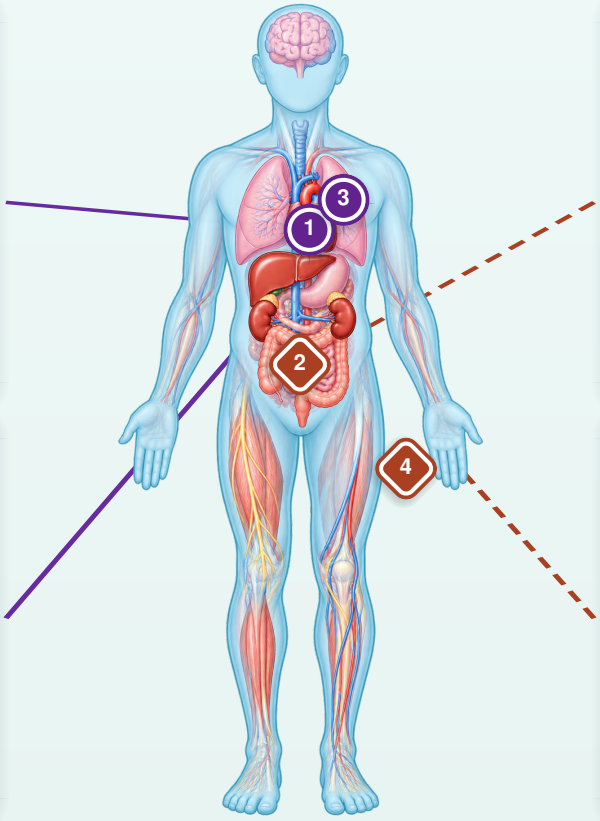
Pertuzumab + trastuzumab

Keep echo appointments; report breathlessness, swelling or reduced exercise tolerance.

2 Diarrhoea

Especially pertuzumab

Report watery stools early, especially with dizziness or poor intake.



3 Infusion reaction

Either antibody

Tell staff immediately about rash, chest tightness or breathing difficulty.

4 Rash / skin

HER2 antibodies

Report persistent or severe skin changes.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

This map covers the two HER2 antibodies only. The selected chemotherapy has additional risks and requires its own patient guide.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Pertuzumab

- Heart pumping-function reduction
- Treatment reaction
- Diarrhoea and rash
- Fatigue

Trastuzumab

- Heart pumping-function reduction
- Treatment reaction
- Diarrhoea and rash
- Fatigue

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Choose the companion chemotherapy

Confirm the exact chemotherapy regimen before issuing its anti-sickness or other supportive prescription. The minimal emetic risk of the two antibodies does not describe the whole combination.

Tests before treatment

full blood count (FBC), kidney and liver profile at baseline and before every 21-day cycle. The team reviews the necessary results before a planned dose. The selected chemotherapy can require extra or more frequent checks.

Keep heart checks on the plan

An echo every 12 weeks is separate from blood tests or cancer imaging. Ask who books and reviews it.

Plan for symptoms

Keep your agreed contact numbers and instructions. New symptoms can need assessment before a routine appointment.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00350/

Sources and review status are available online. Follow your own prescription and hospital instructions.